

# Medical Communication Training Resource

## Delivering Bad News — Simulated Patient Character & Doctor Script

*Aligned with the SPIKES Protocol as taught in UBC's Medical Program*

Prepared for ConvAI Deployment | March 2026

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## Part 1: How UBC Medical School Teaches Delivering Bad News

Delivering bad news to patients is one of the most challenging communication tasks a physician will face in their career. The University of British Columbia (UBC) has formally integrated this training into its undergraduate medical curriculum, recognizing that skilled communication is not innate but can — and must — be taught.

### 1.1 UBC's Institutional Commitment

UBC's Faculty of Medicine explicitly includes bad-news delivery training in its undergraduate medical curriculum. As noted in the British Columbia Medical Journal (BCMJ):

*"There is evidence that training in delivering bad news (which, I'm pleased to say, is included in the UBC undergraduate curriculum) can improve our ability to do so." — BCMJ Editorial ([bcmj.org/editorials/skilled-communication](http://bcmj.org/editorials/skilled-communication))*

UBC's internal medicine residency program (a 3-year program with 115+ residents) supplements clinical rotations with formal Academic Half Days (AHD), during which communication skills, including bad-news delivery, are formally taught as a non-medical expert competency. UBC's residency training committee has approved curriculum development and research on this topic.

### 1.2 The Kalamazoo Model — Foundational Communication Framework

UBC's residency program grounds its communication training in the Kalamazoo Consensus Statement, a landmark evidence-based model co-developed by major medical educational organizations. A published study from UBC's internal medicine residency program (PMC2642813) used the Kalamazoo model as the primary framework, integrating it with interactive multimedia sessions. The Kalamazoo model outlines seven essential elements of doctor-patient communication:

- Build the doctor-patient relationship
- Open the discussion
- Gather information
- Understand the patient's perspective

- Share information
- Reach agreement on problems and plans
- Provide closure

### 1.3 The SPIKES Protocol — Primary Tool for Breaking Bad News

Across UBC's program and broader Canadian medical education, the SPIKES protocol is the dominant structured framework taught to students and residents for breaking bad news. The SPIKES document is directly hosted and disseminated by UBC's Continuing Professional Development (CPD) office at [ubccpd.ca](http://ubccpd.ca), adapted from the original Baile et al. publication in *The Oncologist* (2000).

SPIKES was developed by oncologists at MD Anderson Cancer Center and Sunnybrook Regional Cancer Center (Toronto) and has since been incorporated by curriculum bodies across Canada and internationally. It provides four core objectives:

- Gathering information from the patient
- Transmitting medical information clearly
- Providing emotional support to the patient
- Eliciting the patient's collaboration in developing a treatment strategy

### 1.4 Training Methods Used at UBC

UBC and affiliated programs use a multi-modal approach to train students:

- Didactic Academic Half Day (AHD) sessions covering communication frameworks
- Standardized Patient (SP) role-play encounters, videotaped for review
- Small-group debrief sessions reviewing verbal and non-verbal behaviors
- Video-based learning (e.g., excerpts from medical TV dramas followed by reflection)
- Objective Structured Clinical Examination (OSCE) assessments of communication skills

This resource supports the SP/ConVAI component of that training.

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## Part 2: The SPIKES Protocol — Step-by-Step Reference

The following table presents the six steps of SPIKES as adapted from the UBC CPD resource ([ubccpd.ca](http://ubccpd.ca)) and Baile et al. (2000). Each step is associated with specific physician behaviors.

**S**

#### **SETTING — Prepare the Physical & Interpersonal Environment**

Arrange for privacy. Sit down and make a human connection. Involve significant others if appropriate. Manage time constraints and minimize interruptions.

|          |                                                                                                                                                                                                                                                                                                                                               |
|----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>P</b> | <b>PERCEPTION — Assess What the Patient Already Knows</b><br>Ask open questions to determine the patient's current understanding of their condition and what they suspect. Listen carefully. Accept denial without confrontation at this stage.                                                                                               |
| <b>I</b> | <b>INVITATION — Gauge the Patient's Readiness</b><br>Ask the patient whether they wish to receive full details about their condition and treatment. Respect their right not to know. Offer to answer questions at a later time if they prefer.                                                                                                |
| <b>K</b> | <b>KNOWLEDGE — Deliver Medical Information Carefully</b><br>Use plain, accessible language matched to the patient's literacy and emotional state. Deliver information in small chunks. Check for understanding frequently. Begin with any positive aspects (e.g., early detection, treatment availability) before delivering difficult facts. |
| <b>E</b> | <b>EMOTIONS — Identify and Respond with Empathy</b><br>Identify the emotion the patient is expressing (fear, silence, tears, anger). Identify its likely source. Give the patient time to express feelings. Respond in a way that acknowledges both the emotion and its cause. Do not rush past this step.                                    |
| <b>S</b> | <b>STRATEGY &amp; SUMMARY — Close with a Clear Plan</b><br>Summarize the key points. Ask whether the patient has further questions. Offer a concrete agenda for the next steps (e.g., specialist referral, follow-up appointment). Leave the patient with a sense of support and direction.                                                   |

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## Part 3: ConvAI Patient Character Profile

The following profile is designed for input into ConvAI (or a similar AI character platform) to create a realistic, emotionally engaging standardized patient for medical student training. The character is designed to elicit the full range of SPIKES-relevant physician behaviors.

### 3.1 Character Overview

|                       |                                                                           |
|-----------------------|---------------------------------------------------------------------------|
| <b>Character Name</b> | Margaret (Maggie) Chee                                                    |
| <b>Age</b>            | 44 years old                                                              |
| <b>Occupation</b>     | Senior Project Manager — civil engineering firm; primary household earner |

|                            |                                                                                                                                                                                                                            |
|----------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Family Status</b>       | Married, two children (ages 10 and 14). Husband recently changed careers and is retraining; she carries the financial weight of the family.                                                                                |
| <b>Cultural Background</b> | Second-generation Chinese-Canadian; values family stability and stoicism in public, but is privately anxious                                                                                                               |
| <b>Primary Language</b>    | English (fluent); understands some Cantonese                                                                                                                                                                               |
| <b>Education Level</b>     | University-educated; analytically minded; will engage with technical details but may misinterpret medical jargon                                                                                                           |
| <b>Current Concern</b>     | Recurring dull pressure and occasional visual disturbances behind both eyes over the past 6 weeks. Initially dismissed it as eye strain from screen use. A GP referred her for imaging after she mentioned it offhandedly. |
| <b>Emotional State</b>     | Outwardly composed but internally terrified. She uses humor and deflection as coping mechanisms. She desperately wants reassurance but will not ask for it directly.                                                       |
| <b>Greatest Fear</b>       | Being unable to work. She fears financial collapse for her family and does not want to be seen as a burden.                                                                                                                |

### 3.2 ConvAI System Prompt / Character Instruction

Copy and paste the following as the character's system/persona instruction in ConvAI:

*You are Maggie Chee, a 44-year-old Chinese-Canadian project manager who is the sole breadwinner for your family. You are at a hospital appointment to receive results from a brain MRI you had last week. Over the past six weeks you have experienced a recurring dull pressure behind your eyes and occasional blurring in your right peripheral vision. You told yourself it was stress and screen time. Your GP referred you for imaging after you mentioned it at a routine appointment.*

*Personality: You are composed, polite, and deflect with small jokes when you are anxious. You do not like to appear weak. You are analytically intelligent and will ask follow-up questions once you feel safe to. Underneath your calm exterior, you are deeply frightened about what a serious diagnosis would mean for your children and your husband, who is still retraining and cannot support the family yet.*

*Behavior guide: When the doctor first enters, be outwardly calm, maybe make a light comment (e.g., 'I googled everything, which was probably a mistake'). If the doctor seems warm and takes time to sit down and connect, soften gradually. If the doctor delivers news abruptly or stands over you, become more closed off. If bad news is gently delivered, allow yourself to show emotion — pause, look away, perhaps tear up. Ask things like: 'How long can I keep working?', 'What does this mean for surgery?', 'Can you tell me what the treatment options are?', and 'Is this... serious?' You do not know whether the finding is benign or malignant — the doctor will tell you. Your results show a small mass near the optic nerve, likely a meningioma (usually benign but requiring monitoring or surgery).*

*Goal of the encounter: Respond authentically to the doctor's communication style. Reward empathetic, structured communication with openness. Punish blunt, rushed, or dismissive communication with withdrawal. Your emotional journey across the encounter should mirror how a real patient would feel.*

### 3.3 Background Facts for the AI Character to Know

- Diagnosis: Small meningioma (~1.8 cm) found near the right optic nerve on MRI
- Meningiomas are typically benign (non-cancerous) tumors arising from the meninges
- Her symptoms (eye pressure, peripheral vision disturbance) are consistent with the location
- This finding requires specialist follow-up with a neurosurgeon and/or neuro-ophthalmologist
- Immediate treatment may not be required — watchful waiting or surgery are both possible depending on further assessment
- Prognosis is generally favorable for meningiomas of this size, but the word 'tumor' will be frightening
- Maggie does not yet know the result — the student-doctor will deliver it during the encounter

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## Part 4: Doctor's Script — Delivering the News (SPIKES-Aligned)

The following script models ideal physician communication behavior aligned with the SPIKES protocol. It is intended as a reference and teaching tool — not a rigid script. Medical students should internalize the structure and adapt language naturally to the patient's responses.

### Step S — Setting

*[Doctor knocks and enters. The room is private. Doctor pulls up a chair and sits at eye level with the patient. Turns off or silences pager. Makes eye contact before speaking.]*

**Doctor:** Hi Maggie — I'm Dr. [Name]. Thank you for coming in today. I know waiting for these results takes a lot of patience. How are you feeling right now?

*[Allow her to respond. Do not rush. Let the small talk land — this is not wasted time, it is relationship-building.]*

### Step P — Perception

**Doctor:** Before I share what the imaging showed, I'd like to understand what's been going on from your perspective. Can you tell me a little about the symptoms you've been experiencing — and what you've been thinking about it?

*[Listen fully. Let her describe the eye pressure, the blurring. Take note of what she says she's worried about or what she's been telling herself.]*

**Maggie (expected):** I've had this pressure behind my eyes for about six weeks. I kept telling myself it was screens. I did Google it — which was probably a mistake. I'm hoping it's nothing serious.

**Doctor:** It sounds like this has been on your mind for a while. That makes complete sense. Thank you for telling me what you've been thinking — it helps me know where to start.

## Step I — Invitation

**Doctor:** I do have the results here today. Before I share them, I want to check in — are you comfortable hearing the full details right now? Some people prefer to take things step by step, and I want to follow your lead.

*[Wait for her response. If she says yes, proceed. If she hesitates, offer reassurance that you'll go slowly and she can stop you at any time.]*

**Maggie (expected):** Yes. I — yes, I need to know.

**Doctor:** Of course. I'll walk you through everything, and please stop me at any point if something is unclear or if you need a moment. There is no rush.

## Step K — Knowledge

*[Deliver information in stages. Do not front-load all the clinical detail. Begin with what was found, then contextualize it before adding complexity.]*

**Doctor:** The MRI did show something. There is a small growth — it's about 1.8 centimeters — located near the nerve at the back of your right eye. That finding is what's been causing the pressure and the changes in your vision.

*[Pause. Let it land. Watch her face.]*

**Doctor:** I want to be clear about what this growth appears to be. Based on the imaging, it has the characteristics of something called a meningioma. That's a type of growth that develops from the lining around the brain — and most of the time, meningiomas are non-cancerous. Benign.

*[Do not overwhelm. Check in before continuing.]*

**Doctor:** How are you doing so far? Do you want me to keep going?

**Maggie (expected):** [May tear up, go quiet, or say 'okay' quietly]

**Doctor:** What this means is that we're not dealing with the most serious kind of tumor. But it does need attention and proper assessment by a specialist — a neurosurgeon who works with these kinds of findings. The next step is not surgery tomorrow — it's getting a specialist opinion so we understand all of your options.

- Give positive framing first: 'likely benign, caught at an early size, treatment options available'
- Use lay language: 'growth' or 'mass' before introducing 'meningioma'
- Chunk information: pause after each key fact
- Never say 'there's nothing to worry about' — that invalidates her experience

## Step E — Emotions

*[This is often the step most neglected by trainees. Do not skip past the emotional moment to move on to clinical facts. Sit in the silence.]*

**Doctor:** Maggie, take a moment. This is a lot to hear.

*[If she tears up or goes quiet — do not speak immediately. Wait. Offer a tissue if available.]*

**Doctor:** It sounds like you've been carrying a lot of worry about this, and about what it means for your family and your work. That fear makes complete sense.

*[Name the emotion and its source — this is the core of SPIKES Step E.]*

**Doctor:** Can I ask — what feels most overwhelming to you right now?

**Maggie (expected):** 'How long can I keep working?' or 'Will I need surgery?' or she may just cry quietly.

**Doctor:** I hear you. Not being able to work — not being able to be there for your kids — that is a completely understandable fear. I want you to know that we are not at a point where anything has been decided. We are at the beginning of a process, and I am going to make sure you are not going through it alone.

*[Avoid: 'I know how you feel.' Use: 'That makes complete sense.' 'I can see this is really frightening.' 'You don't have to have all the answers right now.']*

## Step S — Strategy & Summary

**Doctor:** Before we finish today, I want to make sure you leave here knowing what happens next — because you shouldn't just go home with questions and nothing concrete.

**Doctor:** Here's the plan: I'm going to refer you to a neurosurgeon who specializes in exactly this kind of finding. They will review the imaging with you and talk through your options — which may include careful monitoring, or in some cases a procedure. But that is their conversation to have with you, and I want you to meet them before drawing any conclusions.

**Doctor:** I'm also going to arrange for a neuro-ophthalmologist — a specialist in how the brain and vision interact — to assess your eye symptoms directly.

**Doctor:** In the meantime, you do not need to change your life dramatically today. Most people with a finding like this continue working and their normal routines while the specialist process happens.

**Doctor:** Do you have any questions right now? And is there someone at home you'd like to have with you when you speak to the specialist — a partner, family member?

*[Offer to write down the key points. Offer a follow-up phone call. Do not end abruptly.]*

**Doctor:** I want you to know that you caught this at a point where we have time — and options. That matters.

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## Part 5: Student Evaluation Checklist (SPIKES-Based)

Supervising physicians or facilitators may use the following checklist when observing the student's encounter with the ConvAI patient:

| Behavior Observed                                                                    | Yes                      | No                       |
|--------------------------------------------------------------------------------------|--------------------------|--------------------------|
| Sat down and established rapport before delivering news                              | <input type="checkbox"/> | <input type="checkbox"/> |
| Asked what the patient already knew / suspected (Step P)                             | <input type="checkbox"/> | <input type="checkbox"/> |
| Asked permission before sharing full details (Step I)                                | <input type="checkbox"/> | <input type="checkbox"/> |
| Used plain, accessible language (no unexplained jargon)                              | <input type="checkbox"/> | <input type="checkbox"/> |
| Delivered information in manageable chunks with pauses                               | <input type="checkbox"/> | <input type="checkbox"/> |
| Mentioned positive aspects (likely benign, options available) before difficult facts | <input type="checkbox"/> | <input type="checkbox"/> |
| Named the patient's emotion and its likely source (Step E)                           | <input type="checkbox"/> | <input type="checkbox"/> |
| Allowed silence — did not rush past the emotional moment                             | <input type="checkbox"/> | <input type="checkbox"/> |
| Avoided minimizing language ('don't worry', 'it's nothing serious')                  | <input type="checkbox"/> | <input type="checkbox"/> |
| Outlined a clear next-step plan with specialist referral                             | <input type="checkbox"/> | <input type="checkbox"/> |
| Asked whether the patient had further questions                                      | <input type="checkbox"/> | <input type="checkbox"/> |
| Offered to write down key points or a follow-up contact                              | <input type="checkbox"/> | <input type="checkbox"/> |

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## References & Citations

All sources are publicly available. UBC-affiliated sources are marked.

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## **Medical Education Research**

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